

Ipratropium Bromide

(Atrovent)

I. Classification

- Anticholinergic bronchodilator

II. Action(s)

- Inhibits parasympathetic action of acetylcholine in bronchial smooth muscle causing bronchodilation

III. Indications

- Asthma / COPD
- Moderate to severe allergic reaction and anaphylaxis

IV. Contraindications

- Known hypersensitivity / allergy to anticholinergics (Atropine, Ipratropium)

V. Precautions

- The patient's vital signs should be monitored for tachycardia
- May enhance effect or toxicity from other Anticholinergic agents
- Use with caution in Glaucoma, may increase intraocular pressure
- Lung sounds should be auscultated before and after treatment to determine efficacy of treatment.

VI. Drug interactions

- Combination with other Anticholinergic agents may lead to Anticholinergic Toxicity
- Ipratropium bromide increases duration of bronchodilation from Albuterol
- Caution if patient takes MAOIs, tricyclic antidepressants, SSRIs, or Antipsychotics

VII. Adult Dosing and Administration

- Combine with Albuterol for 1st and 2nd dose
- 500 mcg / 2.5 mL

VIII. Pediatric Dosing and Administration

- Combine with Albuterol for 1st and 2nd dose
- 500 mcg / 2.5 mL

IX. Onset

- 15 minutes, Peak effect 1-2 hours

X. Duration

- 4-5 hours

XI. Possible adverse effects

- Hypersensitivity / allergy
- Headache / Dizziness / Blurred Vision
- Nausea / Vomiting / GI upset
- Paradoxical bronchospasm
- Tachycardia / palpitations
- Cough, Shortness of Breath
- Dry throat or mouth, bitter taste in mouth